Mrs. Shanti Prasad

Age / Sex : 60 YRS / F

Referred by : INDRA KIDNEY CARE & SURGICAL CENTER

Reg. no. : 7016

7016

Registered on : 22/10/2026 12:56 PM

Collected on : 22/10/2026 12:00 PM

Received on : 22/10/2026 12:01 PM

Reported on : 22/10/2026 01:00 PM

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HAEMATOLOGY

COMPLETE BLOOD COUNT (CBC)

TEST		VALUE	UNIT	REFERENCE
HEMOGLOBIN	**L**	**10.3**	**g/dl**	**12 - 15**
TOTAL LEUKOCYTE COUNT		6,700	cumm	4,800 - 10,800
DIFFERENTIAL LEUCOCYTE COUNT				
NEUTROPHILS		70	%	40 - 80
LYMPHOCYTE	**L**	**16**	**%**	**20 - 40**
EOSINOPHILS	**H**	**12**	**%**	**1 - 6**
MONOCYTES		02	%	2 - 10
BASOPHILS		00	%	< 2
PLATELET COUNT		2.91	lakhs/cumm	1.5 - 4.1
TOTAL RBC COUNT	**L**	**3.80**	**million/cumm**	**3.9 - 4.8**
HEMATOCRIT VALUE, HCT	**L**	**26.5**	**%**	**36 - 46**
MEAN CORPUSCULAR VOLUME, MCV	**L**	**69.7**	**fL**	**83 - 101**
MEAN CELL HAEMOGLOBIN, MCH		27.1	Pg	27 - 32
MEAN CELL HAEMOGLOBIN CON, MCHC	**H**	**38.9**	**g/dl**	**31.5 - 34.5**
MEAN PLATELET VOLUME, MPV		9.6	fL	6.5 - 12
R.D.W. - SD		44.9	fL	39 - 46
R.D.W. - CV	**H**	**18.7**	**%**	**11.6 - 14**
P-LCR		26.1		19.7 - 42.4
P.D.W.	**H**	**15.6**	**fL**	**9.6 - 15.2**

Possible causes of abnormal parameters:

	High	Low
RBC, Hb, or HCT	Dehydration, polycythemia, shock, chronic hypoxia	Anemia, thalassemia, and other hemoglobinopathies
MCV	Macrocytic anemia, liver disease	Microcytic anemia
WBC	Acute stress, infection, malignancies	Sepsis, marrow hypoplasia
Platelets	Risk of thrombosis	Risk of bleeding

SEROLOGY & IMMUNOLOGY

TEST	VALUE	UNIT	REFERENCE
WIDAL TEST (SLIDE METHOD)			
SALMONELLA TYPHI 'O'	**1:160**		
SALMONELLA TYPHI 'H'	**1:80**		
SALMONELLA TYPHI 'AH'	1:40		
SALMONELLA TYPHI 'BH'	1:20		

C-REACTIVE PROTEIN, CRP (QUANTITATIVE)	0.89	mg/L	< 6 mg/L

BIOCHEMISTRY

LIVER FUNCTION TEST (LFT)

TEST		VALUE	UNIT	REFERENCE
SERUM BILIRUBIN (TOTAL)		0.96	mg/dl	0.2 - 1.2
SERUM BILIRUBIN (DIRECT)		0.17	mg/dl	0 - 0.3
SERUM BILIRUBIN (INDIRECT)		0.79	mg/dl	0.2 - 1
SGOT (AST)		26.87	U/l	0 - 31
SGPT (ALT)		28.96	U/l	7 - 32
SGOT/SGPT Method: Calculated		0.93		
SERUM ALKALINE PHOSPHATASE		93.56	U/l	44 - 147
SERUM PROTEIN	**L**	**6.01**	**g/dl**	**6.4 - 8.3**
SERUM ALBUMIN	**L**	**3.44**	**g/dl**	**3.5 - 5.2**
GLOBULIN		2.57	g/dl	1.8 - 3.6
A/G RATIO		1.34		1.1 - 2.1

LFT Interpretation

Liver Function Blood Test gives an insight into your liver health and helps identify problems like hepatitis, cirrhosis, and fatty liver disease, which may cause similar symptoms but require different treatments to recover.

Test Significance

Besides diagnosing liver problems, LFT's also monitor overall liver functioning. Monitoring helps people with liver disease or taking medication, as it helps screen whether the treatment works fine or requires adjustments. Moreover, Liver Function Tests help determine if someone is at risk of developing liver diseases. Apart from assessing your chances, this test also checks the severity of the liver damage to help the doctor plan and prescribe appropriate treatment.

Increased in: Acute or chronic hepatitis, cirrhosis, biliary tract obstruction, toxic hepatitis, neonatal jaundice (neonatal hyperbilirubinemia), congenital liver enzyme abnormalities (Dubin-Johnson, Rotor, Gilbert, Crigler-Najjar syndromes), fasting, hemolytic disorders. Hepatotoxic drugs.

KIDNEY FUNCTION TEST (KFT)

TEST		VALUE	UNIT	REFERENCE
RANDOM BLOOD SUGAR		101.6	mg/dl	70 - 140
SERUM UREA	**H**	**47.51**	**mg/dl**	**21 - 43**
SERUM CREATININE		1.06	mg/dl	0.55 - 1.13
SERUM PHOSPHORUS		4.44	mg/dl	2.5 - 4.5
SERUM URIC ACID	**H**	**6.57**	**mg/dl**	**2.6 - 6**
SERUM SODIUM		137.5	mmol/L	136 - 146
SERUM POTASSIUM		4.66	mmol/L	3.5 - 5.1
ICALCIUM		1.13	mmol/l	1.13 - 1.33
PH		7.248		

Creatinine is a nitrogenous waste product formed in muscle from creatine phosphate. Endogenous production of creatinine is proportional to muscle mass and body weight.

Causes of Increased Serum Creatinine Level	Causes of Decreased Serum Creatinine Level
1. Pre-renal, renal, and post-renal azotemia 2. Large amount of dietary meat 3. Active acromegaly and gigantism	1. Pregnancy 2. Increasing age (reduction in muscle mass)

~~~ End of report ~~~

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